BASELINE → TREATMENT → FOLLOW-UP

Required safety checks and your individual disease-assessment plan

The team supplies actual dates and resolves any source ambiguity. A reference period does not replace a prescription, a local monitoring plan or an individual treatment decision.

Check	Before starting	During treatment	Between visits	Response / follow-up	After a hold or stop
Bloods and organ safety	Confirm BRAF V600 and treatment setting. Full blood count, kidney/liver tests, creatine kinase (CK) and creatinine, blood pressure, ECG, a heart-pumping assessment by echo or MUGA, and skin examination.	Blood count, kidney/liver tests monthly. CK and creatinine monthly for the first six months, then as indicated. ECG at four weeks, then every 12 weeks and after dose changes; heart-pumping assessment every three months. Skin assessment every 60 days, head/neck examination every three months and chest CT every six months. Response scans follow the individual plan.	Contact the team for new symptoms; extra tests may be needed between scheduled checks.	Safety bloods do not by themselves show whether cancer is responding.	Continue skin surveillance for six months after stopping as specified by the regimen. Agree continuing symptom/organ checks, the next melanoma assessment and medicine-specific pregnancy precautions.
Cancer, skin and daily function	Record diagnosis, stage, current disease sites and the baseline assessment.	Keep the source-specific skin and clinical reviews and the individually agreed response assessments.	Record new lesions, lumps, wound changes, pain and effects on usual activities.	Write the next examination or scan date and the responsible team; no universal scan or tumour-marker interval is assumed.	Follow-up depends on stage, treatment and symptoms. Your team records the skin/lymph-node examination plan, whether body or brain imaging is needed, and which service owns each appointment. A general patient booklet does not set a universal scan interval for every patient.
Medicines and continuing precautions	Review all medicines and the pregnancy/fertility plan.	Review treatment taken or received, holds, adverse effects and supportive medicines.	Keep the actual urgent contacts and do not restart a held medicine yourself.	Bring the medicine list and treatment summary to other services.	Use effective contraception during treatment and for at least one month after the last dose. Hormonal contraception needs an additional or alternative barrier method because interactions can reduce effectiveness. Discuss fertility beforehand and follow the team's breastfeeding instructions. Report a possible pregnancy promptly.

The team arranges and reviews tests. This guide shows expected timing; it does not record completed appointments. NCCP 00563 v4: current regimen source · NCCN Guidelines for Patients: Melanoma, 2026 - supplied education reference

THE WHOLE TREATMENT JOURNEY

Encorafenib and binimetinib for advanced BRAF-mutated melanoma: treatment and checks

NCCP 00563: adults with advanced unresectable or metastatic melanoma with a BRAF V600 mutation.

Take encorafenib once daily and binimetinib twice daily, around 12 hours apart, continuously unless the team tells you to hold them. Treatment continues while beneficial and tolerable. A 28-day review calendar does not mean a routine break or a fixed total number of cycles.

Read from left to right. Columns show stages, not equal lengths of time. This is a general plan: no dates or personal information are needed. Your team confirms which steps apply and arranges appointments.

Treatment, tests and reviews across the course

Care / check	Before starting	During treatment	Between visits	Response / follow-up	After a hold or stop
Bloods and organ safety NCCP 00563	Confirm BRAF V600 and treatment setting. Full blood count, kidney/liver tests, creatine kinase (CK) and creatinine, blood pressure, ECG, a heart-pumping assessment by echo or MUGA, and skin examination.	Blood count, kidney/liver tests monthly. CK and creatinine monthly for the first six months, then as indicated. ECG at four weeks, then every 12 weeks and after dose changes; heart-pumping assessment every three months. Skin assessment every 60 days, head/neck examination every three months and chest CT every six months. Response scans follow the individual plan.	Contact the team for new symptoms; extra tests may be needed between scheduled checks.	Safety bloods do not by themselves show whether cancer is responding.	Continue skin surveillance for six months after stopping as specified by the regimen. Agree continuing symptom/organ checks, the next melanoma assessment and medicine-specific pregnancy precautions.
Cancer, skin and daily function NCCP 00563	Record diagnosis, stage, current disease sites and the baseline assessment.	Keep the source-specific skin and clinical reviews and the individually agreed response assessments.	Record new lesions, lumps, wound changes, pain and effects on usual activities.	Write the next examination or scan date and the responsible team; no universal scan or tumour-marker interval is assumed.	Follow-up depends on stage, treatment and symptoms. Your team records the skin/lymph-node examination plan, whether body or brain imaging is needed, and which service owns each appointment. A general patient booklet does not set a universal scan interval for every patient.

Care / check	Before starting	During treatment	Between visits	Response / follow-up	After a hold or stop
Medicines and continuing precautions NCCP 00563	Review all medicines and the pregnancy/fertility plan.	Review treatment taken or received, holds, adverse effects and supportive medicines.	Keep the actual urgent contacts and do not restart a held medicine yourself.	Bring the medicine list and treatment summary to other services.	Use effective contraception during treatment and for at least one month after the last dose. Hormonal contraception needs an additional or alternative barrier method because interactions can reduce effectiveness. Discuss fertility beforehand and follow the team's breastfeeding instructions. Report a possible pregnancy promptly.

What each check is for

1 Continuous oral treatment

Daily medicines

Encorafenib and binimetinib block two points in the BRAF-MEK growth pathway to help control eligible advanced melanoma. The team reviews response, symptoms and side effects to decide whether the treatment continues.

Ask / act: Confirm the current treatment phase, prescription, tests and support medicines.

Sources: NCCP 00563

At the next review, ask

- Which treatment phase am I in, and what happens next?
- Which test is expected next, who arranges it, and who explains the result?
- Who should I contact if an expected appointment has not arrived?

This map does not show whether a test has been ordered, booked or completed. Do not wait for a scheduled test if you develop new symptoms.

Sources checked 2026-11-06 · Draft source reconciliation; independent clinical, pharmacy and patient/PPI review pending · SACTCheck v0.84.40